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Correct Answer

Next Q

Practice Exam - 1

Question 62 of 117



A 66-year-old with nonischemic cardiomyopathy undergoes heart transplantation. He had no HLA- antibodies prior to transplantation. His post-transplant course has been uncomplicated. Current maintenance immunosuppression includes tacrolimus for goal trough of 10-12 and mycophenolate mofetil 1000 mg BID. He is on aspirin 81 mg daily and atorvastatin 40 mg daily. Corticosteroids were weaned off within the first six months post-transplant.

At 1 year, he has a surveillance coronary angiogram which shows a 40% plaque in the proximal left anterior descending artery and intimal medial thickness of 0.8 mm by IVUS.. Right heart catheterization done at the same time shows normal hemodynamics and cardiac output. Echocardiogram shows normal allograft function. Last LDL was 40 mg/dL. What is the next step in management?

- ☒ **A.** Switch mycophenolate to an mTOR inhibitor (eg, everolimus or sirolimus)
- ☐ **B.** Switch atorvastatin to evolocumab
- ☐ **C.** Consider repeat coronary angiogram in 3 months
- ☐ **D.** Consider evaluation for retransplantation

Commentary References

Testing Point: The board-certified AHFTC specialist should know that in patients with coronary allograft vasculopathy, mTOR inhibitors may delay progression of disease.

Correct Answer: A. Switch mycophenolate to an mTOR inhibitor (eg, everolimus or sirolimus)

Rationale: This patient has cardiac allograft vasculopathy (ISHLT Grade 1). In a study of everolimus versus mycophenolate, patients on everolimus had reduced intimal proliferation at 12 months with similar efficacy to MMF.

Answer B: This patient is already on lipid lowering therapy with good control of LDL,so PSCK inhibitor is not needed.

Answer D: Retransplantation should be considered in patients with CAV 3 with triple vessel disease and/or restrictive hemodynamics and physiology. This patient has less severe disease.

[illegible]

Coronary Allograft Vasculopathy (CAV) Table